This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Abrysvo or Arexvy for active immunisation against Respiratory Syncytial Virus (RSV)

Patient Group Direction, version 008, issued 30 September 2026. Supersedes Version 007, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of the governing guidance for RSV vaccination: Green Book chapter 27a (Respiratory syncytial virus), the JCVI advice of June 2023 and the Abrysvo and Arexvy SmPCs. There is no NICE or NICE CKS topic on RSV vaccination.

Overview

- **RSV** is a common respiratory virus causing **bronchiolitis and pneumonia**, particularly severe in **infants**, **older adults**, and those with **underlying health conditions**.
- Immunisation aims to prevent **severe disease**, hospitalisation, and death.

Immunisation Options

1. Long-acting monoclonal antibody (nirsevimab, Beyfortus): NHS infant programme only, not supplied under this PGD

- Given as a **single intramuscular dose** to provide **passive immunity** for infants.
- **Not a vaccine**, but acts similarly by offering **seasonal protection**.

2. Maternal RSV Vaccine (Abrysvo® – bivalent RSVpreF)

- Approved for **maternal immunisation during pregnancy** to protect newborns via **placental antibody transfer**.
- Offered from 28 weeks of pregnancy, in every pregnancy, all year round (Green Book). Women presenting later can still be vaccinated up to delivery, though the infant gains less passive protection. This PGD's inclusion is 28 to 36 weeks; after 36 weeks refer to the maternity service.
- 3. Older adult RSV vaccines (Arexvy, adjuvanted RSVPreF3, and Abrysvo, bivalent RSVpreF)
- Both are licensed for adults aged 60 years and over as a single 0.5 mL intramuscular dose, and both are supplied under this PGD for that group. Abrysvo is the only one of the two that may be used in pregnancy; Arexvy must not be given to anyone who is pregnant or breastfeeding.

Eligibility (UK Immunisation Programmes)

Infants: nirsevimab (NHS programme, not supplied under this PGD)

- **All infants** born during RSV season (autumn–winter) or entering their first season.
- **High-risk infants**, including:
    - Premature babies (<29 weeks)
    - Infants with chronic lung disease, congenital heart disease, or severe immunodeficiency

Pregnant Women – Abrysvo (Maternal RSV Vaccine)

- Offered from 28 weeks of pregnancy, all year round, to protect infants for their first 6 months of life.

Older Adults

- The NHS older adult programme has run since 1 September 2024 and offers a single dose at 75 to 79 years, following the JCVI advice of June 2023 (adults 75 and over, and 65 to 74 with risk factors). Both vaccines are licensed from 60 years, and this PGD supplies privately from 60 years.

Administration

Product	Dose	Route
Arexvy (adjuvanted RSVPreF3)	Single 0.5 mL dose (60 years and over)	**Intramuscular**
Abrysvo (bivalent RSVpreF)	Single 0.5 mL dose (60 years and over, or pregnancy at 28 to 36 weeks)	**Intramuscular**

- Can be co-administered with other vaccines (e.g. influenza, COVID-19), depending on current UKHSA advice; in older adults RSV vaccine is not routinely scheduled at the same appointment as influenza vaccine (see the Abrysvo cautions).

Contraindications

- Known severe allergic reaction to a previous dose or any component.
- Defer in the case of **acute febrile illness** (minor illness is not a contraindication).

Side Effects

- Generally mild:
  - Local injection site pain or redness
  - Fever, irritability (in infants)
  - Fatigue, headache, myalgia (in adults)

Documentation

- Record **product name, batch number, date, route, site of administration**.
- Report any adverse reactions via the **Yellow Card Scheme**.

Effectiveness

- Both products have demonstrated **high efficacy** in preventing RSV-related hospitalisations and severe disease in **infants**.
- Maternal vaccination reduces infant hospitalisation by up to **80%** in the first 3–6 months of life.

Patient Group Direction

for the supply/administration of

Abrysvo

for vaccination against

Respiratory Syncytial Virus (RSV)

This vaccine is licensed for those who are 60 years and over
and those who are pregnant.

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Abrysvo is indicated for the active immunisation for the prevention of lower tract respiratory disease caused by respiratory syncytial virus (RSV) This vaccine is licensed for those who are 60 years and over and those who are pregnant.
Inclusion criteria	- Adults aged 60 years and over - Pregnant women between 28-36 weeks of gestation - Informed consent obtained - No contraindications to Abrysvo - Where the patient requires immunisation but does not qualify for a free NHS vaccination - Where the patient qualifies for a free NHS vaccination but prefers to have the vaccine privately
Exclusion criteria	- Previous severe allergic reaction to any components of the RSV vaccine - Acute febrile illness (postpone vaccination until recovered). - Those who have already received a complete dose of an RSV vaccine
Cautions	- Observe for 15 minutes post-vaccination. - Advise patients on possible side effects and when to seek medical attention. - Document batch number and expiry date as per local policy. - Use in accordance with the most current national RSV vaccination guidance. - Adrenaline injection 1mg/ml (1:1000) must always be available whenever vaccines are given. Immediate treatment for anaphylaxis should include early administration of intramuscular adrenaline and a call to the emergency services. The healthcare professional delivering the service must be trained to recognise an anaphylactic reaction and be familiar with techniques for resuscitation of a patient with anaphylaxis. - Fainting can occur following, or even before, any vaccination especially in adolescents. It is important that procedures are in place to avoid injury from faints. - Anxiety-related reactions, including vasovagal reactions (syncope), hyperventilation or stress-related reactions may occur in association with vaccination as a psychogenic response to the needle injection. It is important that procedures are in place to avoid injury from fainting. - Abrysvo has not been studied in pregnant individuals below 24 weeks of gestation (SmPC). Under this PGD it must not be given before 28 weeks of gestation. - Advise patients who are immunocompromised that they may have a reduced response to the vaccine. - Administer the vaccine with caution to individuals with any coagulation disorders as there is an increased risk of bleeding following intramuscular administration. A fine needle (23 or 25 gauge) should be

	used for the immunisation, followed by firm pressure applied to the site without rubbing for at least 2 minutes. The individual/parent/carer should be informed about the risk of haematoma from the injection. Do NOT give the vaccine via the subcutaneous route. - Guillain-Barré syndrome has been reported rarely following vaccination with Abrysvo in individuals 60 years and over. Healthcare professionals should be attentive to signs and symptoms of Guillain-Barré syndrome in all Abrysvo recipients to ensure correct diagnosis, in order to initiate adequate supportive care and treatment, and rule out other causes. - Co-administration with other immunisations: • If the vaccine is given at the same time as another vaccine administer at different injection sites, ideally in a different limb. If given in the same limb they should be at least 2.5cm apart. • RSV vaccines can be safely co-administered with shingles vaccines, COVID-19 vaccines and pneumococcal vaccines • Some data indicates that, in older adults, administering Abrysvo at the same time as the seasonal influenza vaccine may reduce the immune response to the RSV vaccine. There is also data that suggests that the response to the influenza A(H3N2) component of seasonal influenza vaccine may be diminished when RSV and seasonal influenza vaccines are co-administered to older adults. It is therefore recommended that RSV vaccine is not routinely scheduled to be given to an older adult at the same appointment or on the same day as an influenza vaccine. But if it is thought that the individual is unlikely to return for a second appointment or immediate protection is necessary, Abrysvo can be administered at the same time as influenza vaccination. • Pregnant women can safely have Abrysvo co-administered with the influenza vaccine and COVID-19 vaccine. There is some data suggesting that co-administration of the RSV vaccine with pertussis-containing vaccines may reduce the response made to the pertussis components. Giving the vaccines separately at the typical scheduled times (around 20 weeks for pertussis and from 28 weeks for RSV) will avoid any potential attenuation of antibody response to the pertussis containing vaccine. If a woman has not received a pertussis containing vaccine by the time she presents for the Abrysvo RSV vaccine, both vaccines can and should be given at the same appointment to provide timely protection against both infections to the infant. • Note - Babies born to women who have had Abrysvo can be safely breastfed.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Abrysvo powder and solvent for solution for injection

Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Intramuscular injection, preferably in the deltoid muscle. • The SmPCs/package inserts should be consulted for preparation steps. • For Abrysvo, Pfizer UK also provides a video of reconstitution steps: pfizerpro.co.uk/medicine/abrysvo/dosing/preparation. • Abrysvo should be administered immediately after reconstitution or within 4 hours if stored between 15°C and 30°C. Chemical and physical in-use stability has been demonstrated for 4 hours between 15°C and 30°C. From a microbiological point of view, the product should be used immediately. If not used immediately, in use storage times and conditions prior to use are the responsibility of the user.
Dose and frequency	Single 0.5 mL dose.
Quantity to be administered and/or supplied	0.5 mL per person as a single dose.
Maximum or minimum treatment period	One-time vaccination per current guidance.
Adverse effects	Pain at injection site, fatigue, headache, muscle ache, fever. Rare: hypersensitivity or allergic reactions. Refer to SmPC. Fainting can occur following, or even before, any vaccination especially in adolescents. It is important that procedures are in place to avoid injury from faints.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows:

	● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	• To seek medical advice if side effects are severe or last more than a few days, and to seek urgent help for any sign of an allergic reaction after leaving the pharmacy. • Advise that no vaccination is 100% effective. • Individuals should be reassured that the inactivated vaccine cannot cause RSV infection. • Advise that the vaccine will not protect against other influenza, Covid or respiratory viruses in current circulation especially during the winter season. • Immunosuppressed individuals should be advised that they may not have a full immune response to the vaccine.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapter 27a Respiratory syncytial virus; JCVI statement on immunisation for RSV, June 2023; Abrysvo and Arexvy Summaries of Product Characteristics, current versions
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD Later versions: see the Version and change record at the end of this document.	15/2/26

Patient Group Direction

for the supply/administration of

Arexvy

for vaccination against

Respiratory Syncytial Virus (RSV)

This vaccine is only licensed for those who are 60 years and over

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Arexvy is indicated for active immunisation for the prevention of lower tract respiratory disease caused by respiratory syncytial virus (RSV) for 60 years and over
Inclusion criteria	- Adults aged 60 years and over - Informed consent obtained - No contraindications to Arexvy - Where the patient requires immunisation but does not qualify for a free NHS vaccination - Where the patient qualifies for a free NHS vaccination but prefers to have the vaccine privately
Exclusion criteria	- Previous severe allergic reaction to any components of the RSV vaccine - Acute febrile illness (postpone vaccination until recovered) - Arexvy should not be administered to those who are pregnant or breastfeeding. There is no evidence that the Arexvy vaccine is safe in these groups. - Those who have already received a complete dose of an RSV vaccine
Cautions	- Observe for 15 minutes post-vaccination. - Advise patients on possible side effects and when to seek medical attention. - Document batch number and expiry date as per local policy. - Use in accordance with the most current national RSV vaccination guidance. - Adrenaline injection 1mg/ml (1:1000) must always be available whenever vaccines are given. Immediate treatment for anaphylaxis should include early administration of intramuscular adrenaline and a call to the emergency services. The healthcare professional delivering the service must be trained to recognise an anaphylactic reaction and be familiar with techniques for resuscitation of a patient with anaphylaxis. - Fainting can occur following, or even before, any vaccination especially in adolescents. It is important that procedures are in place to avoid injury from faints. - Anxiety-related reactions, including vasovagal reactions (syncope), hyperventilation or stress-related reactions may occur in association with vaccination as a psychogenic response to the needle injection. It is important that procedures are in place to avoid injury from fainting. - Advise patients who are immunocompromised that they may have a reduced response to the vaccine. - Administer the vaccine with caution to individuals with any coagulation disorders as there is an increased risk of bleeding following intramuscular administration. - Guillain-Barré syndrome has been reported rarely following vaccination with Arexvy. Healthcare professionals should be attentive to signs and symptoms of Guillain-Barré syndrome in all Arexvy recipients to ensure correct diagnosis, in order to initiate adequate supportive care and

	treatment, and rule out other causes. - Arexvy may be administered concomitantly with COVID-19 vaccine, pneumococcal conjugate vaccine, herpes zoster vaccine and inactivated seasonal influenza vaccine. If Arexvy is to be given at the same time as another injectable vaccine, the vaccines should always be administered at different injection sites.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Arexvy powder and suspension for suspension for injection
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	• Intramuscular injection, preferably in the deltoid muscle. • The SmPCs/package inserts should be consulted for preparation steps and reconstitution. • For Arexvy, GSKPro also provides the reconstitution steps: gskpro.com/en-xg/products/arexvy/administration/ • Do NOT administer the vaccine intravascularly or intradermally. No data is available on the subcutaneous administer of Arexvy. • After reconstitution the chemical and physical in-use stability has been demonstrated for 4 hours at 2°C - 8°C or at room temperature up to 25 °C. From a microbiological point of view, the product should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user and should not be longer than 4 hours.
Dose and frequency	Single 0.5 mL dose.
Quantity to be administered and/or supplied	0.5 mL per person as a single dose.
Maximum or minimum treatment period	One-time vaccination per current guidance.
Adverse effects	- Pain at injection site, fatigue, headache, myalgia, arthralgia. Rare: hypersensitivity reactions. Refer to SmPC. - Adrenaline injection 1mg/ml (1:1000) must always be available whenever vaccines are given. Immediate treatment for anaphylaxis should include early administration of intramuscular adrenaline and a call to the emergency services. The healthcare professional delivering the service must be trained to recognise an anaphylactic reaction and be familiar with techniques for resuscitation of a patient with anaphylaxis. - Fainting can occur following, or even before, any vaccination especially in adolescents. It is important that procedures are in place to avoid

	injury from faints.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	• To seek medical advice if side effects are severe or last more than a few days, and to seek urgent help for any sign of an allergic reaction after leaving the pharmacy. • Advise that no vaccination is 100% effective. • Individuals should be reassured that the inactivated vaccine cannot cause RSV infection. • Advise that the vaccine will not protect against other influenza, Covid or respiratory viruses in current circulation especially during the winter season. • Immunosuppressed individuals should be advised that they may not have a full immune response to the vaccine.

Key references

● NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
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● UKHSA, Immunisation Against Infectious Disease (the Green Book), chapter 27a Respiratory syncytial

virus; JCVI statement on immunisation for RSV, June 2023; Abrysvo and Arexvy Summaries of Product Characteristics, current versions

- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
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30/9/26		31/7/27	

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Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD Later versions: see the Version and change record at the end of this document.	15/2/26

Vaccine safety requirements

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

Cold chain and excursions

Store at +2C to +8C in the original packaging to protect from light. Do not freeze, and discard any vaccine that has been frozen.

COLD CHAIN EXCURSION: any stock exposed to conditions outside +2C to +8C must be QUARANTINED and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use.

Do not administer excursion stock until that assessment is complete and has been recorded.

Where the product SPC gives specific stability data for a temporary excursion, that data governs; otherwise quarantine and assess.

Disposal

Dispose of used syringes, needles, vials and any unused or reconstituted vaccine in a UN-approved puncture-resistant sharps container.

Follow local authority requirements and Health Technical Memorandum 07-01, Safe management of healthcare waste.

Never re-sheath a needle.

Consent in children and young people

Where the individual is UNDER 16, valid consent must come from a person with PARENTAL RESPONSIBILITY, or from the young person where you assess them as GILLICK COMPETENT.

Record which of the two applied. Where consent was given by a person with parental responsibility, record their name and relationship to the patient. Where the young person consented on the basis of Gillick competence, record the basis of that assessment.

A parent accompanying a child does not automatically hold parental responsibility. Ask.

Users must be competent in assessing consent in children and young people before working under this PGD.

Version and change record

Version: v008

Issued: 30 September 2026

Supersedes: Version 007, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v007

Issued: 14 September 2026

Supersedes: Version 006, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v006

Issued: 11 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover title corrected from treatment to active immunisation against RSV.

Guidance summary corrected: nirsevimab marked as an NHS infant product not supplied under this PGD, Arexvy added, the older adult NHS programme stated as running since September 2024, and the administration table now lists Arexvy and Abrysvo with the 0.5 mL dose.

Abrysvo pregnancy caution reworded so that the 24 week SmPC statement and the 28 week PGD limit are two separate facts.

Typos corrected in the Arexvy pregnancy exclusion (There is) and the immunosuppression follow-up advice (may not).

Follow-up advice row replaced the treatment template wording (3 to 4 weeks) with vaccination advice in both arms.

Change history tables now point to the version and change record for versions after 001.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary and references are attributed to the Green Book, JCVI and the SmPCs rather than to a NICE CKS topic that does not exist.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document.

Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Cold chain and excursions, Disposal, Consent in children and young people.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 9 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. The expiry cells read 31 October 2026.

Abrysvo arm: the adrenaline, anaphylaxis-recognition and resuscitation-training requirement that the Arexvy arm carries was absent from the Abrysvo arm. Added.

Version: v004

Issued: 10 September 2026

Supersedes: Version 003, 10 September 2026

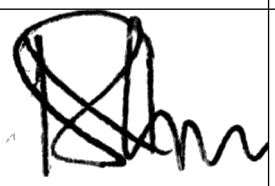
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Guidance summary corrected to the Green Book: the maternal programme is offered from 28 weeks in every pregnancy, all year round, and later vaccination up to delivery is still recommended. The summary previously said 32 to 36 weeks and seasonal, contradicting the PGD's own inclusion of 28 to 36 weeks. Whether supply after 36 weeks should be added to the PGD is recorded for decision.

Signed on behalf of Get Real Health

Version v008, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.